Section 1

HorizonCare Employee Health Plan is a fictional synthetic policy created for dRAGn retrieval demonstrations. It intentionally contains numerical limits, waiting periods, exclusions, claim deadlines, and exceptions that require precise retrieval.

At-a-glance figures

- Hospitalization family-floater limit: ₹5,00,000.
- Hospitalization deductible: ₹10,000.
- Hospitalization co-payment: 10% after deductible.
- Outpatient annual limit: ₹15,000 with 20% member share.
- Maternity annual limit: ₹75,000 with 10% member share.
- Pre-existing disease waiting period: 12 months.
- Maternity waiting period: 9 months.

Operational scenario

A new employee reaches the 30-day eligibility point but has not completed enrollment. Eligibility and enrollment are related but not identical: coverage normally begins after the eligibility date and completion of the enrollment process. A spouse or child added after a qualifying life event should use the applicable special-enrollment window. A dependent child remains covered until the end of the month in which they reach age 25, subject to dependency rules.

Interpretation and operational notes

Health-plan questions should be answered by identifying the benefit category before looking for a numerical limit. Hospitalization, outpatient consultation, prescription medicines, mental health services, ambulance, maternity, and day-care treatment have different rules. The ₹5,00,000 hospitalization limit is a family-floater balance, while the ₹75,000 maternity limit applies to the maternity benefit. Returning the largest number simply because it appears in the document would therefore produce an incorrect answer.

Financial responsibility also has multiple layers. The ₹10,000 hospitalization deductible is a fixed annual amount, while the 10% hospitalization member share is a percentage applied after the deductible. Outpatient treatment uses a 20% member share and does not reduce the hospitalization deductible. Room eligibility is separate again, with a ₹6,000 per-day room limit. A correct answer should preserve these distinctions instead of combining them into one generic co-payment statement.

Claim questions also depend on timing and provider status. Planned hospitalization should normally be pre-authorized 48 hours before admission, while emergency hospitalization should be reported within 24 hours. Reimbursement claims should normally be filed within 30 days of discharge. Network hospitals can provide cashless service subject to authorization; eligible non-network treatment may be reimbursed but is generally paid by the member first. These rules can interact, so retrieval should include enough surrounding context to preserve the condition.

Waiting periods and exclusions are independent from network status. A treatment performed at a network hospital can still be excluded, and a pre-existing condition can still be subject to its waiting period. Maternity has a nine-month waiting period, pre-existing diseases have a 12-month waiting period, and specified procedures have a 12-month waiting period. Accidental injury is treated differently and has no general waiting period from the effective date. This makes the document useful for testing conditional retrieval and citation accuracy.

1. Eligibility and Enrollment

Employee eligibility

- Employees become eligible after 30 days of continuous employment unless their employment letter states otherwise.
- Coverage normally begins after enrollment is completed and the eligibility date is reached.

Dependents

- Spouses and dependent children may be enrolled during annual enrollment or within 30 days of a qualifying life event.
- Dependent children are covered until the end of the month in which they reach age 25, subject to dependency rules.

Changes

A marriage, birth, adoption, or other qualifying life event may create a special enrollment opportunity. Employees should provide the required documentation within the specified enrollment window.

Operational scenario

A family has a ₹5,00,000 hospitalization limit and one member submits a large claim. The amount is a shared family-floater balance, so the first claim reduces what remains for other covered family members during the policy year. This is different from the ₹75,000 maternity limit and the ₹15,000 outpatient limit. A correct answer must identify the benefit category before selecting a number.

Interpretation and operational notes

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2. Coverage Limits

Benefit	Annual limit	Member share
Hospitalization	₹5,00,000	10% after deductible
Outpatient consultation	₹15,000	20%
Prescription medicines	₹20,000	20%
Mental health services	₹25,000	10%
Ambulance	₹10,000	10%
Maternity	₹75,000	10%
Day-care	Within hospitalization limit	10% after deductible

The hospitalization amount is a family-floater balance shared by covered family members. A claim by one member reduces the remaining balance for the policy year.

Operational scenario

Suppose an eligible hospitalization bill is subject to the ₹10,000 annual deductible and the 10% member share. The deductible is a fixed initial amount; the 10% co-payment is a percentage applied after the deductible. Room eligibility is a separate rule with a ₹6,000 daily limit. A member selecting a more expensive room may also face proportionate deductions on related hospital charges, so the room rule should be checked before estimating out-of-pocket cost.

Interpretation and operational notes

Claim questions also depend on timing and provider status. Planned hospitalization should normally be pre-authorized 48 hours before admission, while emergency hospitalization should be reported within 24 hours. Reimbursement claims should normally be filed within 30 days of discharge. Network hospitals can provide cashless service subject to authorization; eligible non-network treatment may be reimbursed but is generally paid by the member first. These rules can interact, so retrieval should include enough surrounding context to preserve the condition.

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Financial responsibility also has multiple layers. The ₹10,000 hospitalization deductible is a fixed annual amount, while the 10% hospitalization member share is a percentage applied after the deductible. Outpatient treatment uses a 20% member share and does not reduce the hospitalization deductible. Room eligibility is separate again, with a ₹6,000 per-day room limit. A correct answer should preserve these distinctions instead of combining them into one generic co-payment statement.

3. Deductibles, Co-payment and Room Limits

Deductible

- The annual hospitalization deductible is ₹10,000 for the family floater.
- The deductible is satisfied before eligible hospitalization coverage applies.

Co-payment

- A 10% co-payment applies to eligible hospitalization expenses after the deductible.
- Outpatient claims have a 20% member share and do not reduce the hospitalization deductible.

Room

- Room charges are covered up to ₹6,000 per day.
- Selecting a room above ₹6,000 per day may result in proportionate deductions to related hospital charges.

The deductible and co-payment should not be confused: the deductible is an initial amount, while the co-payment is a percentage applied afterward.

Operational scenario

A patient is admitted for an emergency at night. The normal pre-authorization rule for planned hospitalization is 48 hours before admission, but emergency hospitalization should instead be reported within 24 hours. A treatment lasting less than 24 hours can still qualify when it is an approved day-care procedure. Pre-authorization is a review of the proposed treatment; it is not a guarantee that every final bill item will be paid.

Interpretation and operational notes

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4. Hospitalization and Day-care

- Planned hospitalization should normally be pre-authorized at least 48 hours before admission.
- Emergency hospitalization should be reported within 24 hours of admission.
- Hospitalization normally requires at least 24 continuous hours unless the procedure is an approved day-care treatment.

Day-care

An approved procedure can qualify as day-care even without a 24-hour admission when it appears in the approved procedure schedule and satisfies the policy conditions.

Authorization scenario

Pre-authorization confirms that the administrator has reviewed the proposed treatment; it does not guarantee payment of every final expense. Final claims remain subject to eligibility, limits, exclusions, documentation, and actual treatment.

Operational scenario

A member asks whether a condition diagnosed during the first few months of coverage is covered. The answer depends on whether it is pre-existing and whether the applicable waiting period has expired. The policy gives pre-existing diseases a 12-month waiting period and maternity a 9-month waiting period. Accidental injury is treated differently and has no general waiting period from the effective date. Waiting periods must therefore be read by condition, not as one universal rule.

Interpretation and operational notes

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5. Waiting Periods

Condition / benefit	Waiting period
Pre-existing diseases	12 months
Maternity	9 months
Specified procedures	12 months
Accidental injury	No general waiting period from effective date

Interpretation

A pre-existing disease is not automatically payable from the first day of coverage. The 12-month waiting period applies unless an explicit policy exception exists. Accidental injuries are treated differently and have no general waiting period from the effective date.

Operational scenario

A cosmetic procedure at a network hospital is not automatically covered merely because the hospital participates in the network. Cosmetic treatment is excluded unless it is medically necessary following an accidental injury or covered reconstruction. Experimental treatment, deliberate self-injury expenses, non-prescription wellness supplements, and personal comfort items are also excluded. Network status and medical eligibility are separate checks.

Interpretation and operational notes

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6. Exclusions

- Cosmetic procedures are excluded unless medically necessary following an accidental injury or covered reconstruction.
- Experimental or unproven treatments are excluded unless explicitly approved by the insurer.
- Expenses caused by deliberate self-injury are excluded.
- Non-prescription wellness supplements are not covered under prescription medicine benefits.
- Personal comfort items, television charges, and non-medical attendant expenses are not covered.
- Treatment that is not medically necessary or outside an eligible benefit category may be rejected.

Why exclusions matter

A treatment may occur in a network hospital and still be excluded. Network status, medical necessity, benefit category, and policy limits are separate checks.

Operational scenario

For a cashless claim, the member presents the health card and identity document at a network hospital, after which the hospital submits the pre-authorization request. For reimbursement, the member submits the claim form, discharge summary, itemized bills, prescriptions, payment proof, and required medical records. Reimbursement claims should normally be filed within 30 days of discharge. Missing documents can pause processing until the requested information is supplied.

Interpretation and operational notes

Claim questions also depend on timing and provider status. Planned hospitalization should normally be pre-authorized 48 hours before admission, while emergency hospitalization should be reported within 24 hours. Reimbursement claims should normally be filed within 30 days of discharge. Network hospitals can provide cashless service subject to authorization; eligible non-network treatment may be reimbursed but is generally paid by the member first. These rules can interact, so retrieval should include enough surrounding context to preserve the condition.

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7. Claims and Pre-authorization

Cashless process

- Present the health card and identity document at a network hospital.
- The hospital sends a pre-authorization request to the claims administrator.
- Treatment proceeds after authorization subject to the approved amount and policy conditions.

Reimbursement process

- Submit the claim form, discharge summary, itemized bills, prescriptions, payment proof, and required medical records.
- Reimbursement claims should normally be submitted within 30 days of discharge.
- Missing documents may pause processing until the requested information is supplied.

Members should retain copies of all submitted documents and claim references so that later queries can be resolved efficiently.

Operational scenario

A member receives treatment at a non-network hospital during an emergency. The treatment may still be eligible for reimbursement, but it is generally not cashless and the member normally pays the hospital first. Maternity has a separate ₹75,000 annual limit and a 9-month waiting period. A newborn should be added within 30 days when the enrollment rule applies. These conditions should be checked separately rather than assuming one hospital rule covers everything.

Interpretation and operational notes

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8. Network Hospitals and Maternity

Network

- Cashless service is available only at participating network hospitals.
- Non-network eligible treatment may be reimbursed, but the member normally pays the hospital first.
- Emergency treatment at a non-network hospital remains subject to notification and policy requirements.

Maternity

- Maternity coverage is limited to ₹75,000 per policy year.
- Normal delivery and medically necessary cesarean delivery are eligible within the maternity limit.
- Maternity has a 9-month waiting period unless a documented exception applies.
- A newborn should be added within 30 days when the enrollment rule applies.

Operational scenario

A mental-health claim can be eligible up to ₹25,000 per year with a 10% member share, while general wellness coaching or gym membership is outside the medical benefit. A ₹7,000 room exceeds the ₹6,000 daily room limit. A pre-existing disease during the first 12 months may be subject to the waiting period, and a reimbursement claim filed 45 days after discharge is outside the normal 30-day window. Each outcome depends on a different policy clause.

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9. Mental Health and Common Claim Scenarios

Mental health

- Eligible mental health treatment is covered up to ₹25,000 per year with a 10% member share.
- General lifestyle coaching, gym memberships, and non-medical wellness programs are not covered under the medical benefit.

Scenarios

A ₹7,000 room exceeds the ₹6,000 eligible room limit and may trigger proportionate deductions. A non-network hospital may be reimbursable but is generally not cashless. A pre-existing disease claim during the first 12 months may be subject to the waiting period. A reimbursement claim submitted 45 days after discharge is outside the normal 30-day filing window and may require an exception.

Operational scenario

The most useful quick-reference numbers are ₹5,00,000 for hospitalization, ₹10,000 for the hospitalization deductible, ₹6,000 for the daily room limit, ₹75,000 for maternity, ₹25,000 for mental health, 12 months for pre-existing disease waiting, 9 months for maternity waiting, and 30 days for normal reimbursement filing. These figures are meaningful only when paired with the benefit category and conditions around them.

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